

1. Administrative & Study Identification

Full Study Title: An Open Label, Multicenter, Dose Escalation, Phase 1 Study to Evaluate Safety/Tolerability, Pharmacokinetics, Pharmacodynamics and Preliminary Anti Tumor Activity of R07247669, a PD1-LAG3 Bispecific Antibody, in Patients With Advanced and/or Metastatic Solid Tumors **Short Title/Acronym:** R07247669 in Metastatic Melanoma and Non-small Cell Lung Cancer and Solid Tumors (NP41300) **Protocol Number:** NP41300 / 2019-000779-18 **NCT Number:** NCT04140500 **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** R07247669 (tobemstomig), a PD-1 and LAG-3 bispecific antibody **Phase of Development:** Phase I **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To establish the maximum tolerated dose (MTD) and/or define the recommended phase 2 dose (RP2D) based on the safety, tolerability, pharmacokinetic (PK), and/or pharmacodynamic (PD) profile of R07247669 in participants with advanced and/or metastatic solid tumors. **Secondary Objectives:** To evaluate preliminary anti-tumor activity (including Objective Response Rate, Duration of Response, and Progression-Free Survival) in participants with selected solid tumors.

2.2 Background & Rationale To address the gap in treatment for patients with advanced solid tumors who have progressed on standard therapies, including standard immunotherapy. Cancer cells can evade the immune system by expressing PD-L1 and utilizing other immune checkpoints like LAG-3. While anti-PD-1/ PD-L1 treatments block the PD-1 pathway, resistance frequently develops. R07247669 is a novel bispecific antibody that simultaneously targets and blocks both PD-1 and LAG-3. By blocking both pathways concurrently, researchers hypothesize that R07247669 will restore T-cell function and show efficacy where prior single-pathway immunotherapies have failed or stopped working.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose-escalation and cohort expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: 170 participants
Number of Sites: Multicenter (including sites in Denmark, Georgia, Israel, Mexico, Singapore) **Estimated Study Start:** 11-Nov-2019 **Estimated Primary Completion:** 30-Jun-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically or cytologically confirmed incurable metastatic disease (advanced and/or metastatic solid tumor malignancies) for which standard curative or palliative measures do not exist, are no longer effective, or are not acceptable to the patient. 2. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 3. For specific expansion cohorts (e.g., NSCLC, Melanoma): Not more than 2 prior lines of treatment for metastatic disease; must have been previously treated with an approved PD-L1/PD-1 inhibitor.

4.2 Exclusion Criteria 1. Pregnancy, lactation, or breastfeeding. 2. Active or untreated central nervous system (CNS) metastases. 3. Positive HIV, hepatitis B, or hepatitis C test result. 4. Active second malignancy or history of autoimmune disease that contraindicates the use of an investigational immunotherapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: R07247669 administered every 2 weeks (Q2W) or every 3 weeks (Q3W) during the dose-escalation (Part A) and expansion (Part B) phases. **Route of Administration:** Intravenous (IV) infusion **Duration of Treatment:** Up to 24 months of treatment, or until disease progression, unacceptable drug toxicity, or participant withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Routine palliative and supportive care for disease-related symptoms. **Prohibited:** Concurrent anti-cancer therapy, investigational agents, live vaccines, and systemic immunosuppressive medications (e.g., high-dose corticosteroids) unless indicated for the treatment of immune-related adverse events (irAEs).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor biopsy/archival tissue collection, Baseline scans
Baseline	Day 1	Cycle 1 Day 1: First IV infusion, PK/PD sampling, Safety Labs
Interim	Every 2 or 3 Weeks	Ongoing infusions matching the schedule, toxicity assessment, immune-related AE (irAE) monitoring, RECIST v1.1 tumor imaging (every 8-12 weeks)
End of Study	Follow-up	

Visit 3 months after the last dose, followed by remote survival and health status checks every 3 months (quarterly) thereafter |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of participants experiencing Dose-Limiting Toxicities (DLTs) during Cycle 1 (Days 1-21 for Q2W dosing, or Days 1-28 for Q3W dosing). Percentage of participants with Adverse Events (AEs) from baseline up to 24 months. **Measurement Method:** Clinical evaluation graded by NCI CTCAE criteria.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR) assessed via RECIST v1.1. 2. Disease Control Rate (DCR), defined as ORR + Stable Disease Rate (SDR). 3. Duration of Response (DOR) and Progression-Free Survival (PFS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous evaluation via site visits and laboratory assessments, focusing specifically on immune-related adverse events (irAEs) commonly associated with checkpoint inhibitors. **Serious Adverse Events (SAEs):** Required to be reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal Dose Escalation Committee (DEC) responsible for reviewing safety data in Part A to clear dose-level escalations and declare the MTD/ RP2D.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: *Safety Set:* All participants who received at least one dose of R07247669 (used for AE and DLT reporting). *Efficacy Evaluable Set:* Participants who received at least one dose and have measurable disease at baseline with at least one post-baseline tumor assessment. **Sample Size Justification:** Approximately 170 participants. The dose-escalation phase uses standard phase I dose-escalation rules (e.g., 3+3 or Bayesian methods) to determine MTD, followed by adaptive expansion cohorts to evaluate preliminary efficacy. **Handling of Missing Data:** Missing safety data will not be imputed. Time-to-event efficacy endpoints (PFS, DOR) will be handled using standard right-censoring methods (e.g., Kaplan-Meier estimates).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine Sponsor Monitoring (onsite and remote) to ensure protocol adherence, accurate data entry, and patient safety, complemented by Dose-Escalation Committee safety reviews.
Audit Trail: Comprehensive maintenance of the Trial Master File (TMF), eCRF audit trails, and strict data clarification form (DCF) generation prior to database locks.

11. Study Identifiers & Governance

Primary ID: NP41300 **Registry Number:** NCT04140500 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** R07247669 in Metastatic Melanoma and Non-small Cell Lung Cancer and Solid Tumors **Official Regulatory Title:** An Open Label, Multicenter, Dose Escalation, Phase 1 Study to Evaluate Safety/Tolerability, Pharmacokinetics, Pharmacodynamics and Preliminary Anti Tumor Activity of R07247669, a PD1-LAG3 Bispecific Antibody, in Patients With Advanced and/or Metastatic Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced and/or Metastatic Solid Tumors
Diagnosis Threshold: Histologically or cytologically confirmed incurable metastatic disease
Medical Methodology: Bispecific Antibody Immunotherapy (PD-1 and LAG-3 blockade)
Optimization Target: Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Phase I Dose Escalation followed by Cohort Expansion
Education Protocol (HCP): Site training on protocol, irAE management, and infusion reactions
Education Protocol (Patient): Informed consent regarding experimental nature, signs of immune toxicity, and reporting requirements
Quality Audit Mechanism: Routine Sponsor Monitoring and Dose-Escalation Committee safety reviews

14. Observation & Follow-Up Schedule

Total Duration: Up to 24 months of treatment + survival follow-up
Onsite Visit Cadence: Every 2 or 3 weeks matching the infusion schedule
Remote Monitoring Frequency: Quarterly (every 3 months) post-treatment for survival check
Checklist Review Interval: Every 8-12 weeks for RECIST tumor response assessment

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					